



DEPARTMENT OF PAEDIATRICS

DISCHARGE SUMMARY

Name	: DHARANESH.T	Age	: 3 yr(s)
Dept	: PAEDIATRICS 2	Sex	: Male
Ward	: PA1C-	Admn Date	: 13-MAR-21
OP No	: O21000094	IP No :	I21007437
		Discharge Date	: 15-MAR-21

DOCTORS

DR NEELAKANDAN MD
PROFESSOR & HOD

DR RAJESH.N.T MD
ASSOCIATE PROFESSOR

DR INDUMATHI.C MD, DCH, MRCPCH (UK)
ASSISTANT PROFESSOR

CASE SUMMARY

:

PRESENTING COMPLAINTS

THIS 3 YEARS 11 MONTHS OLD BOY A CASE OF MENINGOENCEPHALITIC SEQUELAE ON TRACHEOSTOMY AND GASTROSTOMY CAME FOR REVIEW
NO H/O SEIZURES
NO H/O FEVER
NO H/O NEW DEFICITS
NO H/O GI DISTURBANCES

PAST HISTORY

H/O ACUTE MENINGOENCEPHALITIS WITH SEQUELAE / HYPERTENSION DURING JAN 2021.H/O GASTROSTOMY AND TRACHEOSTOMY DURING THAT HOSPITAL ADMISSION

IMMUNISATION

IMMUNISED AS PER NIP SCHEDULE
PNEUMOCOCCAL,INFLUENZA VACCINES NOT GIVEN

DEVELOPMENT

ATTAINED AGE APPROPRIATE DEVELOPEMENTAL MILESTONES BEFORE JAN 2021.FOLLOWING ACUTE MENINGO ENCEPHALITIS AT PRESENT CHILD HAS
GROSS MOTOR:PARTIAL HEAD CONTROL
FINE MOTOR:DOES NOT HOLD OBJECTS
SOCIAL:DOES NOT RECOGNISES PARENTS, RANDOMLY SMILES AT TIMES

FAMILY HISTORY

NO HISTORY OF SIMILAR COMPLAINTS IN THE FAMILY

OTHER RELEVANT HISTORY

:

INVESTIGATION**BioChemistry**

Lab Ref No : 21B048769 Date : 15-MAR-21

PROFILES

Sodium (Plasma/Serum) 139mEq/L.
Potassium (Plasma/Serum) 4.04mEq/L
Chloride (Plasma/Serum) 100mEq/L
Bicarbonate (Plasma/Serum) 22.6mEq/L
Ionised Cal (Plasma/Serum) 1.291m mol/L

Clinical Pathology

Lab Ref No : 21P039199 Date : 15-MAR-21

HAEMATOLOGY

TOTAL RBC COUNT $4.16 \times 10^6/\mu\text{L}$
HEMOGLOBIN 10.6g/dL
HEMATOCRIT (PCV) 33.5%
MCV 80.5fL
MCH 25.4pg
MCHC 31.5g/dl
RDW 19.0%
TOTAL WBC COUNT $7.3 \times 10^3/\mu\text{L}$
NEUTROPHIL % 57.2%
LYMPHOCYTE % 30.0%
MONOCYTE % 10.2%
EOSINOPHIL % 1.9%
BASOPHIL % 0.7%
ABSOLUTE NEUTROPHIL COUNT $4.2 \times 10^3/\mu\text{L}$
ABSOLUTE LYMPHOCYTE COUNT $2.2 \times 10^3/\mu\text{L}$
ABSOLUTE MONOCYTE COUNT $0.7 \times 10^3/\mu\text{L}$
ABSOLUTE EOSINOPHIL COUNT $0.1 \times 10^3/\mu\text{L}$
ABSOLUTE BASOPHIL COUNT $0.1 \times 10^3/\mu\text{L}$
PLATELET COUNT $374 \times 10^3/\mu\text{L}$
MPV 8.4fL
PCT 0.315%
PDW 16.7%

X RAY

: NOT DONE

ECG

: NOT DONE

WEIGHT

KG 12.9CM(ON 3RD CENTILE)

LENGTH: 90CM (LESS THAN 3RD CENTILE)

H.C

47.5CM(BETWEEN 3RD AND 50TH CENTILE)

GENERAL EXAMINATION

ON EXAMINATION,
THE CHILD IS ASLEEP, AFEBRILE, E4 VT M4 GCS:8T/15
PERIPHERIES: WARM
PERIPHERAL PERFUSION GOOD
HYDRATION: GOOD
NO SIGNS OF RESPIRATORY DISTRESS / DEHYDRATION
NO PALLOR / ICTERUS / CYANOSIS / CLUBBING / EDEMA/ LYMPHADENOPATHY

VITAL SIGNS

PR: 130/MIN
BP: 90/60 MMHG
RR:26 /MIN
SPO2: 100 IN RA
TEMP: 98.6F

SYSTEM EXAM

CVS - S1,S2 +, NO MURMURS
RS - BILATERAL NORMAL VESICULAR BREATH SOUNDS
NO WHEEZE / CREPITATIONS
P/A - SOFT, NON TENDER
NO PALPABLE ORGANOMEGALY
CNS -
FLEXURE POSTURE PRESENT
HIGHER MENTAL FUNCTIONS:COULD NOT BE ASSESSED
NOT RECOGNIZES PARENTS

CRANIAL NERVES: I-NOT TESTED , II-B/L PERL 3MM ERTL, III,IV,VI-DEM+LEFT EYE PTOSIS+ , V-TOUCH
SENSATION OVER FACE +, CORNEAL CONJUNCTIVAL REFLEX +, VII-NO OBVIOUS PALSY, VIII-NOT
TESTED , IX,X- GAG REFLEX + , XI,XII- NOT TESTED

MOTOR SYSTEM:

TONE:	RIGHT	LEFT
UPPERLIMB	INCREASED	INCREASED
LOWERLIMB	INCREASED	INCREASED

POWER:	RIGHT	LEFT
UPPERLIMB	3/5	3/5
LOWERLIMB	3/5	3/5

REFLEX:
EXAGGERATED DEEP REFLEXES,ANKLE CLONUS+
B/L PLANTAR-EXTENSOR
NO BLADDER AND BOWEL CONTROL,OTHER SYSTEMS NORMAL
SENSORY:TOUCH SENSATION +
NO SIGNS OF INCREASED ICT.

IN HOSPITAL MEDICATIONS

T.LABETALOL 25MG 1-0-1
T.BACLOFEN 2.5MG 1-1-1

IN HOSPITAL MEDICATIONS

SYP.KEPRA 100MG/ML 3ML-0-3ML

SYP.CALCIMAX 5ML-0-5ML

SYP.ZINCOVIT 5ML-0-5ML

SYP.RANTAC 2.5ML -0-2.5ML

SYP.TRIFER 2.5ML----0----0

ENZOHEAL OINTMENT FOR L/A AROUND TRACHEOSTOMY STOMA SITE

ALLERGIES

NO KNOWN ALLERGY TO FOOD AND MEDICATION

COURSE IN HOSPITAL

: UNEVENTFUL

DISCUSSION

MASTER DHARANESH , A 3 YEAR 11 MONTH OLD BOY A CASE OF POST MENINGITIC SEQUELAE ON TRACHEOSTOMY AND GASTROSTOMY CAME FOR REVIEW.

ON EXAMINATION, THE CHILD WAS AFEBRILE,GENERAL CONDITION FAIR,GCS-8T/15.VITALS WERE STABLE, SYSTEMIC EXAMINATION SHOWED CNS-FLEXED POSTURE,LEFT EYE PTOSIS+,BLINK REFLEX PRESENT,HYPERTONIA IN ALL 4 LIMBS,DTR EXAGGERATED WITH ANKLE CLONUS,B/L PLANTAR-EXTENSOR,NO BLADDER AND BOWEL CONTROL,OTHER SYSTEMS NORMAL.

COMPLETE BLOOD PICTURE SHOWED ANEMIA AND ELECTROLYTES NORMAL. PAEDIATRIC SURGEON OPINION OBTAINED AND GASTROSTOMY BUTTON CHANGED,ENT OPINION OBTAINED AND TRACHEOSTOMY CARE GIVEN.AS HIS BP IS STABLE LABETALOL IS TAPERED AND HE IS ADVISED TO CONTINUE OTHER MEDICATIONS AS ADVISED.

CONDITION AT DISCHARGE

: HEMODYNAMICALLY STABLE

FINAL DIAGNOSIS

POST-MENINGOENCEPHALITIC SEQUELAE

ADVICE ON DISCHARGE

DIET AS ADVISED

REGULAR PHYSIOTHERAPY

TRACHEOSTOMY AND GASTROSTOMY CARE AS ADVISED

MEDICATIONS

T.BACLOFEN 2.5MG 1-----1-----1

SYP.KEPRA 100MG/ML 3ML-----0-----3ML

SYP.CALCIMAX 5ML-----0-----0

SYP.ZINCOVIT 5ML-----0-----0

SYP.TRIFER 0-----0-----3.5 ML

T.LABETALOL 25MG 1-----0-----1/2 X 5 DAYS FOLLOWED BY

MEDICATIONS

T.LABETALOL 25MG 1/2---0-----1/2 X 1 WEEK/ TILL REVIEW

ENZOHEAL OINTMENT FOR L/A AROUND TRACHEOSTOMY STOMA SITE

WHEN TO OBTAIN URGENT CARE

FEVER

VOMITING

SEIZURES

PERSISTENT COUGH

REVIEW DATE

REVIEW IN PAED-2 OPD AT 30.03.2021

SUMMARY PREPARED BY

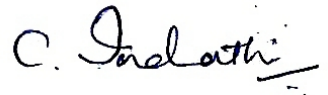
PG:DR.SRINIDHI,DR.ARUNA,DR.MEENU
CRRI-DR.SIRPI

SUMMARY CHECKED BY

DR.INDUMATHI.C
ASSISSTANT PROFESSOR

COPY TO REFERRING DOCTOR

:

DR.INDUMATHI C
(Reg. No. 78851)